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OUTPATIENT CONSULTATION NOTE

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DOCUMENT INFORMATION

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Document Title: Rohan Iyer Neurology Consultation
Document Type: CONSULTATION
Document Date: 18/07/2024
Department: Department of Neurology
Consulting Physician: Dr. Meenakshi Subramaniam, DM Neurology

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PATIENT INFORMATION

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Patient Name: Rohan Iyer
Patient Identifier: NEW-PATIENT-001 (External Synthetic ID)
Date of Birth: 17-Nov-1997 (Age: 26)
Gender: Male
Blood Group: A+
Contact Number: 9876543210
Email: rohan.iyer@fictional.demo
Address: Flat 12B, Orchid Residency, Indiranagar, Bengaluru, Karnataka
Emergency Contact: Priya Iyer (Mother) – 9876543211

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CHIEF COMPLAINT

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Recurrent headaches of approximately four months duration. Patient reports intermittent episodes of moderate-to-severe headache, predominantly unilateral, with occasional sensitivity to light (photophobia).

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HISTORY OF PRESENT ILLNESS

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Mr. Rohan Iyer, a 26-year-old male software engineer, presents to the neurology outpatient clinic with complaints of recurrent headaches that began approximately four months ago (around March 2024). The headaches are described as intermittent, throbbing in character, predominantly affecting the right temporal and frontal regions. Each episode typically lasts between 4 to 12 hours.

The patient reports associated photophobia during the more intense episodes but denies any associated phonophobia, nausea, vomiting, or visual aura. The headaches occur approximately two to three times per week. No identifiable trigger has been reliably established, though the patient notes worsening with prolonged screen exposure and sleep deprivation.

There is no history of head trauma, loss of consciousness, seizures, focal weakness, speech disturbance, diplopia, or gait abnormality. The patient has not experienced any recent febrile illness or neck stiffness.

Over-the-counter analgesics (Paracetamol 500 mg as needed) have provided partial relief. The patient has not received any prior specialist evaluation for headaches.

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PAST MEDICAL HISTORY

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No known major chronic illnesses.
No previous hospitalisations or surgeries.
No known drug allergies.

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FAMILY HISTORY

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Mother: History of episodic migraine headaches (managed conservatively).
Father: No significant medical history.
No family history of epilepsy, stroke, or intracranial pathology.

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PERSONAL HISTORY

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Occupation: Software engineer (desk-based; prolonged screen time)
Diet: Mixed vegetarian and non-vegetarian
Sleep: Approximately 5-6 hours per night (suboptimal)
Smoking: Non-smoker
Alcohol: Occasional (social, approximately 1-2 units per month)
Exercise: Minimal (sedentary lifestyle)
Caffeine: Moderate intake (2-3 cups of coffee per day)

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CURRENT MEDICATION

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Paracetamol 500 mg – oral – as needed (PRN) – self-medicated for headaches.
No prescription medications.

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PHYSICAL EXAMINATION

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General: Well-appearing, alert, oriented, in no acute distress
Vitals:
Blood Pressure: 118/76 mmHg (right arm, sitting)
Heart Rate: 72 bpm, regular
Respiratory Rate: 16/min
Temperature: 36.8°C (oral)
SpO2: 99% on room air
Weight: 74 kg
Height: 175 cm
BMI: 24.2 kg/m²

Head/Neck: Normocephalic, atraumatic. No scalp tenderness.
No cervical lymphadenopathy. Neck supple. No meningismus.
Cardiovascular: S1, S2 heard. No murmur, rub, or gallop.
Respiratory: Bilateral air entry equal. Clear lung fields.
Abdomen: Soft, non-tender. No organomegaly.

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NEUROLOGICAL EXAMINATION

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Mental Status: GCS 15/15. Oriented to time, place, and person.
Speech fluent. No dysarthria or dysphasia.
Cranial Nerves: CN I-XII grossly intact.
Pupils equal, round, reactive to light (PERRL), 3 mm
bilaterally. No papilloedema on fundoscopy.
Visual fields full to confrontation.
Facial symmetry preserved. Hearing intact bilaterally.
Motor: Power 5/5 in all four extremities.
Tone normal. No drift. No fasciculations.
Sensory: Intact to light touch, pinprick, vibration, and
proprioception in all dermatomes tested.
Reflexes: Biceps, triceps, brachioradialis, knee, ankle – 2+
bilaterally and symmetric. Plantar responses flexor
bilaterally.
Coordination: Finger-to-nose and heel-to-shin – normal.
No dysmetria. No intention tremor.
Rapid alternating movements intact.
Gait: Normal gait. Tandem walk normal. Romberg negative.

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ASSESSMENT

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26-year-old male presenting with a four-month history of recurrent episodic headaches with migrainous features (unilateral, throbbing, photophobia). Neurological examination is unremarkable with no focal deficit. The clinical presentation is most consistent with migraine without aura. However, given the relatively recent onset and frequency of headaches, further investigation is warranted to exclude secondary causes.

Differential Diagnosis:

1. Migraine without aura (primary headache) – most likely
2. Tension-type headache – less likely given unilateral and throbbing quality
3. Secondary causes (intracranial pathology) – low clinical suspicion but investigation recommended given relatively recent onset

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PLAN

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1. INVESTIGATIONS:
 - MRI Brain (with and without contrast) – to exclude structural pathology and secondary causes of headache.
 - Routine blood work: CBC, ESR, CRP, Thyroid function (TSH).
2. CONSERVATIVE MANAGEMENT (pending MRI results):
 - Sleep hygiene counselling: aim for 7-8 hours per night.
 - Reduce prolonged screen exposure; take regular breaks (20-20-20 rule).
 - Maintain a headache diary (record frequency, duration, triggers, severity).
 - Adequate hydration.
 - Limit caffeine to 1-2 cups per day.
3. ACUTE TREATMENT:
 - Continue Paracetamol 500 mg PRN (max 2g/day) for acute episodes.
 - If Paracetamol is insufficient, may use Ibuprofen 400 mg PRN (if no contraindications). Advise against frequent analgesic use (>10 days/month) to prevent medication-overuse headache.

4. FOLLOW-UP:

- Review after MRI Brain with the imaging report.
- If headache frequency persists or worsens, consider preventive therapy at next visit.
- Return sooner if: new neurological symptoms, sudden severe headache, visual changes, seizures, or progressive worsening.

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PHYSICIAN SIGNATURE

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Electronically signed by:

Dr. Meenakshi Subramaniam, DM Neurology
Senior Consultant, Department of Neurology
Sanjeevan Neurosciences Clinic, Bengaluru

Date: 18/07/2024

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